Supreme Court, U.S.
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No. 97-53

In The

Supreme Court of the United States

October Term, 1997

JANE M. ROBERTS,
Guardian for Wanda Y. Johnson,

Petitioner,

vs.

GALEN OF VIRGINIA, INC., Formerly d/b/a
Humana Hospital – University of Louisville,

Respondent.

On Writ Of Certiorari
To The United States Court Of Appeals
For The Sixth Circuit

BRIEF OF PETITIONER

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QUESTION PRESENTED

Whether a claim under the Emergency Medical Treatment and Active Labor Act, 42 U.S.C. § 1395dd, requires proof of an improper motive on the part of the hospital, its staff or physicians as a prerequisite for recovery.

TABLE OF CONTENTS

Page

QUESTION PRESENTED..... i

TABLE OF AUTHORITIES..... iii

OPINIONS BELOW..... 1

JURISDICTION..... 1

STATUTORY PROVISIONS INVOLVED..... 1

STATEMENT OF THE CASE..... 2

SUMMARY OF THE ARGUMENT..... 14

ARGUMENT..... 17

PROOF OF IMPROPER MOTIVE ON THE PART OF THE HOSPITAL IS NOT A PREREQUISITE FOR RECOVERY IN A CLAIM UNDER THE EMERGENCY MEDICAL TREATMENT AND ACTIVE LABOR ACT, 42 U.S.C. § 1395dd..... 17

A. The Plain Reading of the Statute Compels the Conclusion That Improper Motive is Not a Required Element of an EMTALA Transfer Violation Claim..... 17

B. Proof of an Improper Motive to Establish a Claim Under EMTALA Has Been Expressly or Implicitly Rejected By Every Circuit Court of Appeals Which Has Considered the Issue..... 28

CONCLUSION..... 34

STATUTORY APPENDIX.....A1

TABLE OF AUTHORITIES

Page

CASES

Brooker v. Desert Hosp. Corp., 947 F.2d 412 (9th Cir. 1991)..... 19

Brooks v. Maryland General Hosp., Inc., 996 F.2d 708 (4th Cir. 1993)..... 19

Burditt v. U.S. Dept. Of Health and Human Services, 934 F.2d 1362 (5th Cir. 1991).....16, 28, 29

Chevron U.S.A., Inc. v. Natural Resources Defense Council, Inc., 467 U.S. 837 (1984)..... 26

City of Chicago v. Environmental Defense Fund, 511 U.S. 328 (1994).....25, 26

Cleland v. Bronson Health Care Group, Inc., 917 F.2d 266 (6th Cir. 1990).....18, 19, 22, 29, 32

Collins v. DePaul Hospital, 963 F.2d 303 (10th Cir. 1992)..... 19

Correa v. Hospital San Francisco, 69 F.3d 1184 (1st Cir. 1995), *cert. denied*, 517 U.S. 1136 (1996)..... 32

Eberhardt v. City of Los Angeles, 62 F.3d 1253 (9th Cir. 1995)..... 33

Gatewood v. Washington Health Care Corp., 933 F.2d 1037 (D.C. Cir. 1991).....19, 30, 32

Holcomb v. Monahan, 30 F.3d 116 (11th Cir. 1994).... 33

Keene Corp. v. United States, 508 U.S. 200 (1993)..... 26

Marshall v. East Carroll Parish Hosp. Serv. Dist., 134 F.3d 319 (5th Cir. 1998)..... 33

Martin v. OSHRC, 499 U.S. 144 (1991)..... 26

McDonnell-Douglas v. Green, 411 U.S. 792 (1973)..... 27

TABLE OF AUTHORITIES – Continued

Page

Power v. Arlington Hosp. Ass'n, 42 F.3d 851 (4th Cir. 1994) 18, 30, 32

Repp v. Anadarko Mun. Hosp., 43 F.3d 519 (10th Cir. 1994) 33

Roberts v. Galen of Virginia, Inc., 111 F.3d 405 (6th Cir. 1997) 22

Smith v. Richmond Memorial Hospital, 243 Va. 445, 416 S.E.2d 689 (1992) 18

Texas Dept. of Community Affairs v. Burdine, 450 U.S. 248 (1981) 27

STATUTES

28 U.S.C. § 1254(1) 1

42 U.S.C. § 1395dd 1, 17

42 U.S.C. § 1395dd(a) ..... *passim*

42 U.S.C. § 1395dd(b) 16, 23, 28

42 U.S.C. § 1395dd(b)(1) 12, 20

42 U.S.C. § 1395dd(b)(1)(A) 12

42 U.S.C. § 1395dd(c)(2) 12, 21

42 U.S.C. § 1395dd(d) 12, 25

42 U.S.C. § 1395dd(d)(3) 12, 26

42 U.S.C. § 1395dd(d)(3)(A) 12

42 U.S.C. § 1395dd(e)(1) 12, 20

42 U.S.C. § 1395dd(e)(3)(A) 12, 20

42 U.S.C. § 1395dd(e)(3)(B) 12, 21

TABLE OF AUTHORITIES – Continued

Page

42 U.S.C. § 1395dd(e)(4) 12, 21

42 U.S.C. § 1395dd(e)(4)(A) 12

42 U.S.C. § 2000 26

42 U.S.C. § 2000e-2(a)(1) 26, 27

Ky. Rev. Stat. § 387.530 9

Ky. Rev. Stat. § 387.740 9

PUBLICATIONS

Paul Starr, *The Social Transformation of American Medicine* (1982) 18

Trieger, Note, *Preventing Patient Dumping: Sharpening the Cobra's Fangs*, 61 N.Y.U.L. Rev. 1187 (1986) 17

MISCELLANEOUS

131 Cong. Rec. S. 13,904 (Daily Ed. Oct. 23, 1985) 25

59 Fed. Reg. 32,086, 32,104 (1994) 26

OPINIONS BELOW

The opinion of the United States Court of Appeals for the Sixth Circuit is reported at 111 F.3d 405 (6th Cir. 1997) and is reprinted in the Appendix of the Petition for Writ of Certiorari previously filed herein at A1-A25. The two opinions of the United States District Court for the Western District of Kentucky are not reported but are reprinted in the Appendix to the Petition for Writ of Certiorari at A26-A45 and A48-A52, respectively.

JURISDICTION

The order of the United States Court of Appeals for the Sixth Circuit was entered on April 9, 1997 affirming the order of the United States District Court for the Western District of Kentucky which granted summary judgment in favor of the Respondent. This Court has jurisdiction under 28 U.S.C. § 1254(1).

STATUTORY PROVISIONS INVOLVED

The statute involved in this case is the "Emergency Medical Treatment and Active Labor Act," 42 U.S.C. § 1395dd, reprinted in the Appendix to this Brief and in the Appendix to the Petition for Writ of Certiorari at A55-A65.

STATEMENT OF THE CASE

The Petitioner, Wanda Y. Johnson (hereinafter "Johnson"), is a 40 year old mother of one child who was run over by a truck while walking home from her job as a cashier for Hardees Restaurant in Bardstown, Kentucky, late on the evening of May 19, 1992. (Deposition of Wanda Johnson, hereinafter "Johnson Depo., R. 50" at 17-18)[1]. She was flown from Bardstown to the Respondent's facility, Humana Hospital – University of Louisville (hereinafter "Humana") where she was diagnosed with serious fractures to her pelvis, right leg and four vertebrae of her spine as well as a brain injury. (Deposition of Walid Abou-Jaoude, M.D., hereinafter "Abou-Jaoude Depo., R. 67" at Exhibit 2). Johnson underwent exploratory surgery upon admission to Humana during which her spleen was removed. (*Id.*). A tracheostomy was surgically inserted into her neck and she was hooked to a ventilator to support her breathing. (*Id.*). Johnson's medical condition remained volatile throughout her hospitalization at Humana as she experienced numerous complications, including recurrent urinary tract infections and multiple pneumonias. (J.A. at

[1] References to items contained in the Joint Appendix are designated "J.A." with the page in the Joint Appendix where the item may be found. References to items contained in the Appendix to the Petition for Writ of Certiorari are designated "P.A." with the page in the Petition's Appendix where the item may be found. Relevant portions of deposition testimony not reproduced in the Joint Appendix are referenced to the specific deposition transcript and page number and docket number in the Record where the particular transcript may be found, i.e., "R. 50" would refer the Court to docket entry 50 to find the transcript of Wanda Johnson's Deposition.

116-117, *id.* at 14-15). Her hospital course was further complicated by fever and overwhelming infection with an elevated temperature to as high as 108 degrees. (Abou-Jaoude Depo. at Exhibit 2). During her hospitalization, Johnson's neurologic status deteriorated to a state of flaccid quadriparesis. (J.A. at 116, *id.* at 15). Despite Humana's treatment, Johnson's temperature spikes could not be controlled and the episodes of pneumonia and urinary tract infection continued through the last hours of her hospitalization. (J.A. at 126-128, *id.* at 49-50; Exhibit 2).

Despite the volatility of her condition, in early July, 1992, Johnson's physicians requested that the social services department at Humana begin making contacts with rehabilitation facilities and nursing homes to explore potential placement for Johnson if and when her condition was stabilized and she was ready to leave an acute care setting. (J.A. at 118-120, Abou-Jaoude Depo., R. 67 at 18-20; Deposition of Michele Weaver, R. 45 at 8).

The social workers who work at Humana are employed directly by Humana. Their job includes making placement at nursing homes and rehabilitation facilities for Humana patients who will not be able to return home following their hospitalization. (Deposition of Nancy Fred, hereinafter "Fred Depo., R. 10" at 4-5). The social workers are also called upon to seek legal intervention for patients who are incapable of independent decision making. (*Id.* at 5).

Humana's social workers immediately experienced difficulty in their efforts to find potential placement facilities for Johnson. The social workers learned that Johnson's husband had abandoned their five year old son and was cohabitating with another woman in Springfield, Kentucky. (*Id.* at 14-15, 23). Although the husband occasionally called the hospital, he would not leave a telephone number or contact location which was reliable enough for the social workers to maintain a consistent dialogue in developing a discharge plan. (*Id.* at 23-24). Furthermore, Johnson's mother was elderly and was herself ill and unable to function independently. (*Id.* at 23). No other close family member existed to provide assistance to the Humana social workers in developing a discharge plan for Johnson. Jane Roberts, Johnson's aunt, occasionally visited the hospital and attempted to locate Johnson's husband for the social workers. (J.A. at 19; *id.* at 24-25). However, Roberts had not been appointed as Johnson's guardian at that time and did not have the legal authority, nor had she been requested, to act as Johnson's surrogate decision maker. (Deposition of Jane Roberts, hereinafter "Roberts Depo., R. 51" at 21).

When Humana's social workers began their search for a placement for Johnson, she still could not breathe without assistance of a mechanical ventilator. (Fred Depo., R. 10 at 11-12). The ventilator dependency limited placement options. (*Id.*). However, days before her discharge, Humana unhooked Johnson from the ventilator and Humana's social workers contacted Chandler Nursing Home near Evansville, Indiana on July 20, 1992 as a possible placement facility. (J.A. at 19-23; *id.* at 25-28). Based on clinical information provided by Humana,

Chandler Nursing Home accepted Johnson conditioned upon on-site evaluation and approval by its Director of Nursing. (*Id.*). However, on July 22, 1992, only hours before the planned transfer, Chandler's Director of Nursing rescinded the nursing home's acceptance of Johnson following her personal evaluation. (J.A. at 23; *id.* at 28). Although Chandler was licensed as a skilled facility, its Director of Nursing determined that Johnson's condition was too serious for the facility to provide adequate care. (J.A. at 23-24; *id.* at 29). Chandler's decision to deny Johnson's placement was an obvious setback for Humana's social workers. However, they still expected within days to receive approval from a physical rehabilitation facility which had been previously contacted by Humana social worker Nancy Fred. On July 20, 1992, Fred "predicted" in her progress notes that approval by that facility, New Medco Rehabilitation Center, would take only another one to two weeks. (J.A. at 19; *id.* at 25). New Medco's expected acceptance of Johnson must have been encouraging to Fred since it was her opinion that a rehabilitation facility was more appropriate to treat Johnson's condition than was a nursing home. (*Id.* at 53).

Another significant complication in locating a facility which would accept Johnson was her inability to pay. (*Id.* at 16-18). By mid-July, 1992, Johnson's application for Kentucky Medicaid benefits had still not been approved. Johnson had no private health insurance and obviously did not have the resources to compensate any facility for her care. (Roberts Depo., R. 51 at 29; J.A. at 38-39; Fred Depo., R. 10 at 55). Johnson's medical expenses at Humana had already approached $400,000.00 and it was obvious to Humana that Johnson would never be able to

compensate it for those charges. (Deposition of John Nord, hereinafter "Nord Depo., R. 47" at 18-23). However, regardless of Johnson's inability to pay, Humana was obligated to provide treatment to her as part of a contractual relationship between Humana, the Commonwealth of Kentucky, the City of Louisville, Kentucky and Jefferson County, Kentucky whereby Humana received millions of dollars in subsidies in exchange for its agreement to provide services at no charge to Kentucky patients who did not qualify for Medicaid, who had no other resources and who were without private health insurance. (*Id.*).

In the midst of Humana's placement dilemma and shortly after Chandler had denied acceptance of Johnson, Joanna Nolte, a marketer for Hometown Living Centers ("Hometown") in Indianapolis, Indiana, visited Louisville in an effort to initiate a relationship with the Humana hospitals whereby Humana would place its patients in Hometown's Indianapolis nursing homes. (J.A. at 26-27, 41-42; Fred Depo., R. 10 at 31; Deposition of Joanna Nolte, hereinafter "Nolte Depo., R. 18" at 22).

On the evening of July 22, 1992, Nolte met with members of Humana's Social Services Department. (*Id.*). Among those who attended Nolte's presentation was Nancy Fred, who had taken charge of Johnson's discharge plan. (*Id.*). Nolte was neither a doctor nor a nurse. (Nolte Depo., R. 18 at 4). Her job was to locate potential residents for her employer's nursing homes and to refer those potential residents to administrators and directors of nursing at the respective facilities. (*Id.* at 16-17). Nolte explained that Hometown operated several nursing

homes in Indianapolis, including Mayfair Manor ("Mayfair") and Crestview Nursing Home ("Crestview"). (J.A. at 27-28; Fred Depo., R. 10 at 32). At the conclusion of Nolte's presentation, she was approached by Nancy Fred who requested that Nolte look at Johnson and consider accepting Johnson in one of the Hometown facilities. (J.A. at 42-43; Nolte Depo., R. 18 at 23-24). According to Nolte, Fred appeared to be in a real placement dilemma with Johnson. (J.A. at 48; *id.* at 31). Nolte testified that Fred complained that she was receiving pressure from the administration at Humana to find a placement for Johnson because of Johnson's inability to pay for her care. (J.A. 44-46; *id.* at 26-27). Nolte explained to Fred that Johnson's level of acuity exceeded her own unilateral authority to grant approval. However, Nolte agreed to refer Fred's request to the Administrator and Director of Nursing at Mayfair for evaluation. (J.A. 46-47; *id.* at 29-30).

All the while, Johnson's medical condition continued to be volatile. On July 23, 1992, her temperature rose to 101 degrees. (J.A. at 126-128; Abou-Jaoude Depo., R. 67 at 49-51; Exhibit "A"). A sputum culture was abnormal and her doctor suspected that she had yet another episode of urinary tract infection. (*Id.*). On the evening before her discharge, Johnson's right lung collapsed and treatment was- required every two hours. (Deposition of Donna Kaelin, R. 48 at 16). The morning of Johnson's discharge, her white blood count was over twice the normal level, her temperature had been rising for two days, and the nurses noticed cloudy urine, thick yellow sputum and an elevated heart rate, all indicating the presence of infection. (Deposition of Karen Martin, hereinafter "Martin

Depo., R. 9" at 14-19; Deposition of Suzanne Griffith, R. 49 at 9-10). The nurse on duty during Johnson's last day at Humana felt strongly enough about Johnson's condition to specifically document in her nursing notes that she had called the elevated white blood count and temprature as well as the cloudy urine to the attention of the attending physician. (Martin Depo., R. 9 at 21-23).

Meanwhile, Mayfair declined to accept Johnson because her care would require too many skilled nursing hours. (J.A. at 50; Nolte Depo., R. 18 at 33-34). However, on that same day Crestview, which had fewer skilled nurses than Mayfair, agreed to accept her. (*Id.*). The on-duty nurse at Humana learned at about midday on July 24, 1992 that Johnson would be transferred to Crestview. (Martin Depo., R. 9 at 7). In fact, the first notation that Crestview had even been considered by the social workers at Humana appeared in a social services note on July 24, 1992, wherein Nancy Fred indicated that Johnson would be transferred to Crestview on that date at 2:30 p.m. (Patient Progress Notes dated July 24, 1992 attached to Fred Depo., R. 10 at Exhibit "L").

In its haste to expedite Johnson's transfer to Crestview, Humana ignored its standard procedures and protocols for discharge and transfer. (J.A. at 11-18, 33-34; Fred Depo., R. 10 at 41-42; Policy Procedure 768-901 and 768-912). No authorization for the transfer was obtained from either Johnson's husband or mother. (*Id.*). Furthermore, although Johnson's condition had clearly deteriorated during the previous two days and Johnson wasn't even competent to understand or participate in her own discharge plan, Humana's social workers made no effort

to obtain court intervention and have a guardian or surrogate decision maker appointed for Johnson.[2] (J.A. at 22, 116; Fred Depo., R. 10 at 27-28; Abou-Jaoude Depo., R. 67 at 15). Instead, Humana unilaterally decided to ship Johnson to Indianapolis and Nancy Fred simply called Jane Roberts, Johnson's aunt and occasional visitor, ***told her*** that Johnson was being transferred to Crestview Nursing Home that afternoon and provided Roberts Crestview's telephone number. (Roberts Depo., R. 51 at 22-23, 42-43). Fred did not ask Roberts to check with Johnson's family to determine whether they would like to visit the facility before the transfer was arranged or whether they would even approve of a transfer which would place Johnson in a different state and some three hours driving distance from their home. (J.A. at 33-36; Fred Depo., R. 10 at 41-43). Fred gave Roberts no information about Crestview or about any benefits it could provide to Johnson. (Roberts Depo., R. 51 at 23-24). In fact, neither Fred nor anyone else at Humana could then provide any such information because they knew almost nothing about Crestview. (J.A. at 27-29; Fred Depo., R. 10 at 32-33, 48-49).

Fred didn't know and had failed to ask Nolte if Crestview had ever handled a patient with Johnson's level of acuity. (*Id.*). Fred also didn't know and again failed to ask whether or not Crestview had ever been

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[2] Any interested person or entity is permitted by Ky. Rev. Stat. § 387.530 to file a petition for determination of disability and appointment of a guardian in Kentucky's District Courts. The courts are permitted to take emergency action where necessary by Ky. Rev. Stat. § 387.740.

cited for deficiencies in the care of residents. (*Id.* at 40). Despite the fact that Humana had never previously placed a patient at Crestview and despite the fact that it made no inquiries to gather even basic information, it decided to ship Johnson to Crestview and thereby rid itself of a patient who placed a serious financial drain on the hospital.

Even more significant than Humana's violation of its own discharge policy mandating that social workers advise patients or family of discharge plans in order to provide them at least an opportunity to decline the discharge and explore other arrangements, was Humana's violation of its discharge policy by failing to gain approval for the transfer from the accepting physician at Crestview. (Roberts Depo., R. 51 at 43; Abou-Jaoude Depo., R. 67 at 35-36; J.A. 6-10; Policy Procedure 0420). Crestview's doctors were unaware of Johnson's impending transfer and in fact knew nothing at all about her and her medical condition until she had already arrived. (J.A. at 80-81; Deposition of Kambiz T. Karimi, M.D., hereinafter "Karimi Depo., R. 89" at 12; Deposition of Taiseer Shatara, M.D., hereinafter "Shatara Depo., R. 90" at 10-12).

No reasonable person could argue that Crestview was an appropriate placement for Johnson. Even Crestview's own physicians admit that Crestview was inappropriate and indicated that had they known all the relevant information about Johnson's condition they would not have accepted her at Crestview. (J.A. at 81-82; Karimi Depo., R. 89 at 13; J.A. at 88-91; Shatara Depo., R. 90 at 23-26).

Johnson's condition deteriorated significantly as a result of her transfer from Humana. Upon Johnson's arrival, the nurses at Crestview were alarmed by Johnson's condition and they believed she was going to die. (J.A. at 58-59; Deposition of Charna Y. Treser, hereinafter "Treser Depo., R. 92" at 66-67). Within hours, Johnson's temperature spiked to 105 degrees and she was rushed to nearby Midwest Medical Center where she remained hospitalized for almost six months. (J.A. at 62; Deposition of John W. Stuy, M.D., hereinafter "Stuy Depo., R. 88" at 7, 71). Dr. John W. Stuy, Johnson's principal treating physician during her hospitalization in Indianapolis, testified that the complications experienced by Johnson in Indianapolis predated her transfer and were exacerbated because of her transfer to a facility which was unable to properly care for her. (J.A. at 62-69, 76-79; *id.* at 7-14, 15-17, 27-31). Humana had transferred Johnson before it had received the full lab reports relative to the infection suffered by her during the last day of her hospitalization in Louisville and simply had prescribed a broad spectrum antibiotic hoping it would fight off the infection. (P.A. at A13; J.A. at 128; Abou-Jaoude Depo., R. 67 at 51). According to Dr. Stuy, the antibiotic prescribed for Johnson when she left Humana was not sensitive to the lab cultures grown at Midwest Medical Center, indicating that the antibiotic was not treating her infection. (J.A. at 62-69, 76-79; Stuy Depo. at 7-14, 15-17, 27-31). His opinion was that if she had remained on the wrong antibiotic much longer, she may well have died. (*Id.*). Dr. Stuy emphatically stated that Johnson should never have been transferred from Humana. (*Id.*).

In addition to imperiling Johnson's life and health, Humana's hurried transfer of Johnson to Indianapolis placed her in fiscal peril because she was ineligible for public medical assistance in Indiana. (Stipulations dated April 20, 1995, R. 62). Had she remained hospitalized at Humana, Johnson would not have incurred personal liability for the payment of hundreds upon hundreds of thousands of dollars in medical expenses because of Humana's contract with Kentucky governmental entities which paid it millions of dollars annually to provide care to indigent patients. (Nord Depo., R. 47 at 18-23). Unfortunately for Johnson, once she was transferred from Humana to Indianapolis she was no longer entitled to free medical care. Adding insult to injury, Johnson's application for Medicaid benefits in Indiana was denied because she was not a resident of Indiana. (Stipulation dated April 20, 1995, R. 62). As a direct result of Humana's unilateral decision to ship Johnson to Indiana, Johnson incurred and is now personally obligated to pay medical expenses in excess of $388,000.00. (*Id.*).

In October, 1992, Roberts was appointed as Johnson's Guardian and shortly thereafter Roberts initiated this action against Humana alleging that Humana had violated the provisions of the Emergency Medical Treatment and Active Labor Act, 42 U.S.C. § 1395dd, *et seq.* (hereinafter "EMTALA") by transferring Johnson to Crestview before stabilizing her condition. After voluminous discovery, Humana moved the District Court for Summary Judgment on the EMTALA claim. (R. 63). In response to Humana's Motion for Summary Judgment, the District Court determined that the relevant issues were, "whether Johnson was stabilized when she was transferred to

Crestview and whether the Hospital acted with an improper motive." (P.A. at A29; Memorandum Opinion entered November 13, 1995, R. 73 at 3). The District Court concluded that genuine issues of material fact existed as to both of those issues and overruled Humana's Motion for Summary Judgment. (P.A. at A46-A47; Order entered November 13, 1995, R. 74).

However, Humana subsequently moved the District Court to reconsider that Opinion, arguing that EMTALA required Johnson to prove that the decision of the surgical resident who discharged Johnson was also "improperly motivated." (R. 78). The District Court agreed, citing affidavits of Humana doctors stating that they believed Johnson was stable when she was transferred and that Johnson's financial condition had no effect on their decision to transfer her. (J.A. at 131-137). The District Court held that Johnson's failure to produce any significant evidence in opposition to the affidavits of the Humana doctors meant she had failed to meet her burden to establish a genuine issue of material fact with respect to whether her transfer was "caused" by an improper motive. (P.A. at A50-A51; Memorandum Opinion entered February 15, 1996, R. 83).

On appeal, Johnson pointed out to the Court of Appeals for the Sixth Circuit that every other circuit court of appeals in the country which had considered the issue had expressly rejected the notion that proof of improper motive was required to establish a claim under EMTALA. However, in a 2 to 1 decision, the court chose to stand alone, holding that EMTALA required proof of improper motive to establish a claim. Straying even further from the unanimous holdings by the other circuits, it also held

that even proof of improper motive by hospital administration is insufficient to maintain an EMTALA claim and required proof that the discharging physician himself was also improperly motivated. (P.A. at A1-A25).

Johnson filed her Petition for Writ of Certiorari to this Court on July 8, 1997 and the Order of the United States Supreme Court granting certiorari on the question of whether improper motive is a required element of proof under EMTALA was entered on June 8, 1998.

SUMMARY OF ARGUMENT

A. Although EMTALA was enacted in response to the widespread perception that hospitals were "dumping" indigent or uninsured patients for purely economic reasons, the statute as ultimately passed by Congress provides protections to "any individual" who comes to an emergency department or hospital seeking emergency medical treatment and is denied needed care. 42 U.S.C. § 1395dd(a), (b)(1). This expansive language has been unanimously recognized by the Circuit Courts of Appeal, including the Sixth Circuit. However, the Sixth Circuit has substantially limited EMTALA's practical application by requiring not only proof of violation of the statute but also further proof that the cause of the statutory violation was an improper motive by the hospital.

The Sixth Circuit's injection of an improper motive requirement is not based upon any express language in EMTALA. In fact, the plain reading of EMTALA compels a conclusion clearly contrary to that reached by the Sixth Circuit but in accord with the holdings of every other

circuit court of appeals in the country which has considered the issue. EMTALA contains no motive requirement. The clear Congressional purpose of EMTALA was to deter patient dumping and to compensate victims of dumping. An interpretation of EMTALA which places the significant burden upon claimants to establish improper motive would undermine EMTALA's purpose.

Furthermore, Johnson's interpretation of EMTALA that no proof of improper motive is required is buttressed by the absence of a fault requirement in the civil enforcement subsection of Section 1395dd(d). While a neighboring subparagraph requires the Secretary of Health and Human Services to prove that hospitals and doctors have negligently violated EMTALA before civil monetary penalties can be imposed, no such fault requirement is contained in the subparagraph permitting civil enforcement of EMTALA by harmed individuals. This conspicuous absence of a fault requirement in the private cause of action subsection of EMTALA demonstrates that Congress did not intend to saddle an individual with having to prove fault in order to recover under EMTALA. The Secretary of Health and Human Services, who administers the civil monetary fine program created by EMTALA, also interprets EMTALA as containing no requirement that a claimant prove some "impermissible motive" on the part of the hospital. In short, there is no statutory basis for imposing a requirement on claimants under EMTALA to prove that a hospital's violation of the statute was the result of some improper motive, and imposing such a burden would thwart the attainment of the Act's goals.

B. The Sixth Circuit stands alone in its injection of a requirement that a claimant under EMTALA prove that the offending hospital acted with an improper motive. The Fifth Circuit expressly rejected the argument that proof of an improper motive was required under EMTALA in *Burditt v. U.S. Dept. Of Health and Human Services*, 934 F.2d 1362 (5th Cir. 1991). *Burditt*, like the case at bar, involved an EMTALA claim under Section 1395dd(b) that a patient was improperly transferred from a hospital before the patient's condition had been stabilized.

Three other Circuits have rejected the notion that a claimant is required to prove an improper motive on the part of the hospital in cases dealing with the "appropriate medical screening examination" section of EMTALA. The District of Columbia Circuit, the Fourth Circuit and the First Circuit have specifically considered the Sixth Circuit's reasoning and expressly rejected it, holding that proof of an improper motive is not required under EMTALA. Additionally, while not specifically citing the Sixth Circuit's position or addressing a claim that improper motive must be shown, four other circuits have reached conclusions that are inconsistent with the Sixth Circuit's construction of EMTALA. These courts have recognized that it is impermissible to unilaterally change the plain reading of EMTALA to impose proof requirements which have no statutory basis.

ARGUMENT

Proof Of Improper Motive On The Part Of The Hospital Is Not A Prerequisite For Recovery In A Claim Under The Emergency Medical Treatment And Active Labor Act, 42 U.S.C. § 1395dd.

This case raises a fundamental question of federal law regarding the character of proof necessary to establish a claim under the Emergency Medical Treatment and Active Labor Act, 42 U.S.C. § 1395dd (hereinafter "EMTALA"). This Court is urged to construe the statute as plainly written, adopting the reasoning of every circuit court except the Sixth which has considered the issue and determined that EMTALA does not require proof of an improper motive on the part of the hospital as a prerequisite for recovery.

A. The Plain Reading of the Statute Compels the Conclusion That Improper Motive is Not a Required Element of an EMTALA Transfer Violation Claim.

In 1986, Congress passed EMTALA with bipartisan support and President Ronald Reagan signed the legislation into law. The Act, also referred to as the "anti-dumping" provisions of the Social Security Act, was enacted in response to the widespread perception that hospitals were "dumping"[3] indigent or uninsured patients by either denying them emergency care or transferring them to other institutions for purely economic

[3] For a thorough review of the patient dumping problem see Trieger, Note, *Preventing Patient Dumping: Sharpening the Cobra's Fangs*, 61 N.Y.U.L. Rev. 1187 (1986).

reasons.[4] *See, e.g., Power v. Arlington Hosp. Ass'n*, 42 F.3d 851, 856 (4th Cir. 1994), *Cleland v. Bronson Health Care Group, Inc.*, 917 F.2d 266, 269 (6th Cir. 1990). However, patient dumping is not limited to a refusal to provide emergency room treatment. As recognized by the Supreme Court of Virginia:

> It occurs, and is equally reprehensible, at any time a hospital determines that a patient's condition may result in substantial medical costs and the hospital transfers the patient because it fears it will not be paid for those expenses.

Smith v. Richmond Memorial Hospital, 243 Va. 445, 452, 416 S.E.2d 689, 692 (1992).

Although the evolution of the statute was fueled by Congress's desire to prohibit hospitals from dumping indigent or uninsured patients, the statute as ultimately passed by Congress provides protections to far more patients. Rather than limiting the protections provided by EMTALA to indigent or uninsured individuals, application of the statutory protections against patient dumping is extended to, "any individual (whether or not eligible

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[4] Scholars agree that the fundamental change which has occurred in institutional health care in this country is somewhat responsible for the patient dumping problem. "Hospitals were originally built primarily to care for the poor. As hospitals began treating more paying patients, new hospitals were established as business enterprises in the late nineteenth century. In the twentieth century, hospitals have become as competitive as any commercial enterprise." Paul Starr, *The Social Transformation of American Medicine*, 145-148 (1982).

for benefits under [Medicare])" who comes to an emergency department or hospital. 42 U.S.C. § 1395dd(a), (b)(1).

The federal circuit courts of appeals which have addressed the scope of EMTALA, including the Sixth Circuit, have been unanimous in recognizing the expansive language of EMTALA and have rightly extended its application to all emergency patients rather than limiting the Act's protections only to indigent patients.[5] EMTALA provides these individuals with three distinct protections. First, a hospital must provide for an "appropriate medical screening examination within the capability of the hospital's emergency department" to any individual who comes to the emergency department if a request is made on the individual's behalf for examination or treatment for a medical condition. 42 U.S.C. § 1395dd(a). Second, if

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[5] See *Brooks v. Maryland General Hosp., Inc.*, 996 F.2d 708, 711 (4th Cir. 1993) ("the language of the Act does not require a showing that a claimant is uninsured or indigent, nor does it provide that the hospital breaches the Act's duties only when it acts with economic motives"); *Cleland v. Bronson Health Care Group, Inc.*, 917 F.2d 266, 268 (6th Cir. 1990) ("we hold Congress to its words, that this statute applies to any and all patients"); *Brooker v. Desert Hosp. Corp.*, 947 F.2d 412, 415 (9th Cir. 1991) ("we hold that the Act applies to any and all patients, not just patients with insufficient resources"); *Collins v. DePaul Hospital*, 963 F.2d 303, 308 (10th Cir. 1992) ("the fact that Congress . . . viewed [EMTALA] as a so-called 'anti-dumping' bill . . . does not subtract from its use of the broad term 'any individual' "); *Gatewood v. Washington Health Care Corp.*, 933 F.2d 1037, 1040 (D.C. Cir. 1991) ("though the . . . Act's legislative history reflects an unmistakable concern with the treatment of uninsured patients, the Act itself draws no distinction between persons with and without insurance").

a medical screening examination reveals that an "emergency medical condition"[6] exists, the hospital must then provide the patient with treatment necessary to "stabilize"[7] this condition. 42 U.S.C. § 1395dd(b)(1). Finally, the hospital is prohibited from effecting a "transfer"[8] of the

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[6] 42 U.S.C. § 1395dd(e)(1) provides that the term "emergency medical condition" means –

> "(A) a medical condition manifesting itself by acute symptoms of sufficient severity (including severe pain) such that the absence of immediate medical attention could reasonably be expected to result in –
>
> (i) placing the health of the individual (or, with respect to a pregnant woman, the health of the woman or her unborn child) in serious jeopardy,
>
> (ii) serious impairment to bodily functions, or
>
> (iii) serious dysfunction of any bodily organ or part; or
>
> (B) with respect to a pregnant women [woman] who is having contractions –
>
> (i) that there is inadequate time to effect a safe transfer to another hospital before delivery, or
>
> (ii) that transfer may pose a threat to the health or safety of the woman or the unborn child."

[7] 42 U.S.C. § 1395dd(e)(3)(A) provides that the term "to stabilize" means, "with respect to an emergency medical condition described in paragraph (1) (A), to provide such medical treatment of the condition as may be necessary to assure, within reasonable medical probability, that no material deterioration of the condition is likely to result or occur during the transfer of the individual from a facility, or, with respect to an emergency medical condition described in paragraph (1) (B), to deliver (including the placenta)."

[8] 42 U.S.C. § 1395dd(e)(4) provides that the term "transfer" means "the movement (including the discharge) of an individual outside a hospital's facilities at the direction of any person employed by (or affiliated or associated, directly or indirectly, with) the hospital, but does not include such a movement of an individual who (A) has been declared dead, or (B) leaves the facility without the permission of any such person."

individual from the hospital if the emergency medical condition has not been "stabilized"[9] unless rigid statutory requirements are met by the hospital.[10]

EMTALA, as passed by Congress, does not contain an express requirement that an improper or non-medical motive be proven to establish a claim for violation and no such requirement can reasonably be implied from the language of the statute. Nor does Humana base its argument on the Act's express language. Further, no court of appeals, other than the Sixth Circuit, has imposed such a requirement on any litigant. The Sixth Circuit has substantially limited EMTALA's practical application by inventing a proof requirement found nowhere in the statute. The Sixth Circuit has determined that in addition to

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[9] 42 U.S.C. § 1395dd(e)(3)(B) provides that the term "stabilized" means, "with respect to an emergency medical condition described in paragraph (1)(A), that no material deterioration of the condition is likely, within reasonable medical probability, to result from or occur during the transfer of the individual from a facility, or, with respect to an emergency medical condition described in paragraph (1)(B), that the woman has delivered (including the placenta)."

[10] Humana does not contend that the "appropriate transfer" provisions of 42 U.S.C. § 1395dd(c)(2) apply in this case.

showing a violation of the statute, an improper, or non-medical, motive must also be proven as an element of EMTALA for both inappropriate screening and transfer claims. *Roberts v. Galen of Virginia, Inc.*, 111 F.3d 405 (6th Cir. 1997); *Cleland v. Bronson Health Care Group, Inc.*, 917 F.2d 266 (6th Cir. 1990).

The "improper or non-medical motive" requirement was first espoused by the Sixth Circuit in *Cleland*. *Cleland* involved an allegedly inappropriate screening examination under Section 1395dd(a). In that case, the Clelands took their fifteen year old son to the hospital emergency room complaining of cramps and vomiting. During the medical screening examination, the child's condition was misdiagnosed as influenza and the child was sent home. Less than twenty-four hours later, the child suffered cardiac arrest and died.

In analyzing EMTALA, the Sixth Circuit determined that Congress had limited "the cause of action provided by the Act to only those who did not receive an 'appropriate' screening or who were not 'stabilized' before being transferred or discharged." *Cleland*, 917 F.2d at 271. The court determined that the term "appropriate" used by Congress in describing the required medical screening examination of Section 1395dd(a) was ambiguous. It purportedly looked to legislative history, along with other aids to construction, and determined that in the context of EMTALA, " 'appropriate' must . . . correctly be interpreted to refer to the motives with which the hospital acts. If it acts in the same manner as it would have for the usual paying patient, then the screening provided is 'appropriate' within the meaning of the statute." *Id.* at 272.

Extending its statutory misinterpretation to the next illogical step, the Sixth Circuit in *Cleland* as well as in the case at bar reasoned that the same improper or non-medical motive requirement that it had extrapolated from its construction of the adjective "appropriate" in the medical screening requirement of 42 U.S.C. § 1395dd(a) also applied to the Section 1395dd(b) prohibition against transferring unstabilized patients, "though with even greater force." *Id.* at 271. However, the Sixth Circuit's analysis fails as a textual matter because the term "appropriate" contained in the medical screening requirement of the statute, and upon which the Sixth Circuit based its entire analysis, is found nowhere in the section of the statute which requires necessary stabilizing treatment.[11] Rather than taking notice of the absence of the key word "appropriate," the Sixth Circuit's analysis proceeds as if the Congressional omission has no significance. In fact, the Sixth Circuit did not even purport to rely upon any

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[11] The Sixth Circuit's exercise in statutory misinterpretation did not stop there. It next ignored the fact that the statute imposes direct rather than vicarious liability on Humana and determined that Johnson's showing of an improper non-medical motive on the part of hospital administration and social services staff was *not* sufficient if the independent contractor surgical resident who signed Johnson's discharge was not also implicated in the improper conduct. It is clear that, even if improper motive is a required element of proof under EMTALA, Johnson has established sufficient evidence of improper motive on the part of Humana employees or administration to withstand Summary Judgment. Therefore, even if this Court determines that proof of an improper motive is required, it should nevertheless reverse the decision of the Sixth Circuit in this case with directions to remand the case to the District Court for trial.

express language in § 1395dd(b) that could be construed to impose an improper motive requirement for failure to stabilize and improper transfer claims.

A proper reading of the statute considers the stabilization requirement in its entire context and compels a conclusion clearly contrary to that reached by the Sixth Circuit and consistent with the holdings of the other circuits. The plain terms of the statute require proof only of a violation, causation and damages. EMTALA contains no motive requirement. The statute requires only that Johnson show that she was transferred from Humana's hospital and that her emergency medical condition had not been stabilized at the time of transfer.

The evidence adduced thus far certainly is sufficient to allow a trier of fact to conclude that a statutory violation occurred. Johnson was transferred while she had an active infection and just hours after her lung had collapsed. Rather than waiting a few additional hours for full lab reports to be completed so that an appropriate medication could be prescribed, Humana did little more than pull an antibiotic out of the medicine cabinet and hope it would work. Humana's effort to rid itself of Johnson almost cost Johnson her life and certainly caused material deterioration in her condition. For this reason, the District Court determined that Johnson was entitled to a jury trial on the issue of whether she was stable when transferred if improper motive was not a required additional element of proof. That finding was not challenged by Humana on appeal. (P.A. at A3-A4, A13-A14).

Johnson's assertion that recovery under the transfer and stabilization provisions of EMTALA is not predicated

on motive is consistent with the Congressional purpose of EMTALA to deter patient dumping and to compensate victims of dumping. The purpose of EMTALA was "to send a clear signal to the hospital community . . . that all Americans, regardless of wealth or status, should know that a hospital will provide what services it can when they are truly in physical distress." 131 Cong. Rec. S. 13,904 (Daily Ed. Oct. 23, 1985) (Statement of Senator Durenberger).

This interpretation is buttressed by the absence of a fault requirement in the civil enforcement subsection of Section 1395dd(d). Subparagraph (1) provides for civil monetary penalties imposed by the Secretary of Health and Human Services ("Secretary") against hospitals and doctors who ***negligently*** violate EMTALA. Subparagraph (2) of § 1395dd(d) allows civil enforcement of EMTALA to any "individual who suffers personal harm as a direct result of a participating hospital's violation of a requirement of this section. . . . "

Conspicuously absent from the civil enforcement provisions of the statute is any requirement that a harmed individual prove that a hospital's violation was intentional, knowing, or even negligent. The omission of a fault requirement in the private right of action provision when its neighboring subsection contains such a requirement is a clear indication that the omission was intentional and suggests that a harmed individual can prevail on a lesser showing than would be required of the Secretary in imposing a civil penalty. "It is generally presumed that Congress acts intentionally and purposely" when it "includes particular language in one section of a statute but omits it in another." *City of Chicago*

v. Environmental Defense Fund, 511 U.S. 328, 338 (1994) (citing *Keene Corp. v. United States*, 508 U.S. 200, 208 (1993)). Clearly, the express requirement of negligent conduct for civil penalties and the lack of such an express requirement for civil enforcement does not support a conclusion that a private plaintiff must make the greater showing of improper motive required by the Sixth Circuit in this case.

The Secretary of Health and Humana Services has also concluded that proof of improper motive is not an essential element of an EMTALA violation claim. In the preamble to Interim Final Rules and Regulations governing the Department's Health Care Financing Administration and its Office of the Inspector General, the Secretary stated: "Also with regard to appropriate transfers, we note that the Secretary has taken the position that in proving that a hospital or physician violated [42 U.S.C. § 1395dd(b)], there is no requirement to prove that the transfer was effected due to some 'impermissible motive.' " 59 Fed. Reg. 32,086, 32,104 (1994). HHS administers the civil monetary fine program created by EMTALA. *See* 42 U.S.C. § 1395dd(d)(3). Accordingly, because HHS's interpretation is reasonable, it deserves deference. *See Martin v. OSHRC*, 499 U.S. 144 (1991); *Chevron U.S.A., Inc. v. Natural Resources Defense Council, Inc.*, 467 U.S. 837, 842-845 (1984).

The total absence of any requirement in EMTALA that motive be proven demonstrates Congress' intent to spare patients the burden of proving they were denied treatment because of intentional discrimination based on some improper or non-medical motive. In contrast, statutes like Title VII of the Civil Rights Act of 1964, 42 U.S.C. §§ 2000e *et seq.*, which prohibits discrimination in

employment "*because of*" an "individual's race, color, religion, sex, or national origin," *id.* at § 2000e-2(a)(1) (emphasis added), make clear that Congress knows how to write an intent or effects-based law when it is so inclined.

Furthermore, Congress' decision to make motive irrelevant is entirely logical. As this Court well knows from Title VII cases, motive and intent are always difficult to prove. *See Texas Dept. of Community Affairs v. Burdine*, 450 U.S. 248 (1981); *McDonnell-Douglas v. Green*, 411 U.S. 792, 805 (1973). Those standards often create vexing proof problems, as evidence of discriminatory intent is rarely set forth in documents or other records. Congress understood that hospitals and physicians involved in dumping would have no trouble advancing plausible-sounding medical explanations for their conduct which, in fact, would be pretexts to disguise violations of EMTALA nor would they leave a paper trail of documents evidencing their improper motive. Congress clearly intended not to saddle patients, physicians, hospitals and the courts with the time-consuming, expensive, fact-intensive battles that are inevitable whenever a *Burdine*-type inquiry is required. There is no reason to believe that Congress intended to add to patients' burden by requiring them to prove improper motives as well as violations of EMTALA, particularly where Congress' sole concern was to ensure access to appropriate emergency medical treatment for all.

B. Proof of an Improper Motive to Establish a Claim Under EMTALA Has Been Expressly or Implicitly Rejected By Every Other Circuit Court of Appeals Which Has Considered the Issue.

The Sixth Circuit stands alone in its injection of a requirement that a claimant under EMTALA prove that the offending hospital acted with an improper motive. As a result, claimants in the Sixth Circuit must prove more than their counterparts anywhere else in the country. The statute has thus been so narrowed that it may be virtually impossible for an EMTALA claimant to prevail in the Sixth Circuit. Recognizing that the court cannot impose an element necessary for a cause of action under EMTALA where the statute makes no such provision, the other circuit courts of appeals have rejected improper or non-medical motive as a requirement of proof under EMTALA.

The Fifth Circuit Court of Appeals expressly rejected the argument that proof of an improper motive was required under EMTALA in *Burditt v. U.S. Dept. of Health & Human Serv's*, 934 F.2d 1362 (5th Cir. 1991). *Burditt*, like the case at bar, involved an EMTALA claim under § 1395dd(b) that a patient was improperly transferred from a hospital before the patient's condition had been stabilized. In that case, a physician transferred a patient in active labor to another hospital without insuring that the patient's condition was "stabilized." The Secretary imposed a civil penalty against the physician. The Fifth Circuit affirmed the civil penalty in spite of the physician's argument that the Secretary was required to prove

that he had acted with an improper motive. In rejecting this assertion, the Fifth Circuit stated as follows:

> Burditt asks this court to invent a requirement found nowhere in the statute that an improper, or non-medical, motive for transfer must be proved as an element of all EMTALA transfer violations. As written, EMTALA prevents patient dumping without such a requirement. . . . We refuse to alter the statutory scheme.

Burditt, 934 F.2d at 1373 (internal citation omitted).

Other circuits have rejected the notion that a claimant is required to prove an improper motive on the part of the hospital in cases dealing with the "appropriate medical screening examination" section of EMTALA. In *Gatewood v. Washington Health Care Corp.*, 933 F.2d 1037 (D.C. Cir. 1991), the District of Columbia Circuit became the first appellate court to expressly reject the Sixth Circuit's reasoning in *Cleland*. In that case, Mrs. Gatewood's husband died after he was discharged from the Defendant hospital's emergency room. She brought an action under EMTALA claiming that the hospital misdiagnosed her husband's condition in its emergency room. The trial court granted the hospital summary judgment noting that Mrs. Gatewood's allegations of misdiagnosis, without more, were simply not actionable under EMTALA.

In specifically rejecting the Sixth Circuit's reasoning in *Cleland* and in specifically holding that improper motive is not a prerequisite to recovery under even the medical screening portion of the statute, the D.C. Circuit stated as follows:

> In our view, the hospital fulfills the "appropriate medical screening" requirement when it conforms in its treatment of a particular patient to its standard screening procedures. By the same token, any departure from standard screening procedures constitutes inappropriate screening in violation of the Emergency Act. The motive for such departure is not important to this analysis, which applies whenever and for whatever reason a patient is denied the same level of care provided others and guaranteed him or her by subsection 1395dd(a).

Gatewood, 933 F.2d at 1041.

The reasoning of the District of Columbia Circuit was followed, and that of the Sixth Circuit again repudiated, by the Fourth Circuit in *Power v. Arlington Hosp. Ass'n*, 42 F.3d 851 (4th Cir. 1994). In that case Mrs. Power came to the Arlington Hospital emergency room complaining of pain in her hip, her lower abdomen, and in her back running down her leg, and reporting that she was unable to walk, was shaking and had severe chills. After being discharged with directions to take pain medication and call an orthopedist if the pain got worse, Power returned to the emergency room the next day and was diagnosed as suffering from septic shock and was immediately admitted to the intensive care unit of the hospital where she remained for over four months.

Mrs. Power brought an action against Arlington Hospital claiming that she had not received an appropriate medical screening examination on her first visit to the hospital and upon subsequent admission that she was transferred before her condition had been stabilized. She did not contend, and was unable to offer proof, that the

hospital's failure to provide an appropriate medical screening examination or that her transfer before stabilization had been based on any improper motive. The case was tried before a jury and a multi-million dollar verdict was rendered in favor of Mrs. Power on the medical screening count of her complaint but in favor of the hospital on her claim that she had been transferred in an unstable condition. On appeal of the verdict, the Fourth Circuit rejected the hospital's argument that Power was required to prove improper motive to succeed on her EMTALA claim, explaining as follows:

> First, there is nothing in the statute itself that requires proof of indigence, inability to pay or any other improper motive on the part of a hospital as a prerequisite to recovery. The language of subsection 1395dd(a) simply refers to "any individual" who presents to the emergency room. Second, it seems to us that the expanse of motives suggested by the Sixth Circuit in *Cleland* is so broad as to be no limit at all, and as a practical matter amounts to not having a motive requirement. Anyone who alleges that she did not receive an appropriate medical screening examination can simply find an improper motive that fits, whether it is sex, nationality, income or occupation, and simply allege it. Which leads, thirdly, to the most fundamental problem with the motive requirement: the proof predicament. We agree with Power's position that having to prove the existence of an improper motive on the part of a hospital, its employees or its physicians, would make a civil EMTALA claim virtually impossible. We do not believe that proving the inner thoughts and

> prejudices of attending hospital personnel is required in order to recover under EMTALA.

Power, 42 F.3d at 857-858.

The First Circuit, too, has rejected the *Cleland* position and followed the District of Columbia and Fourth Circuits. *Correa v. Hospital San Francisco*, 69 F.3d 1184 (1st Cir. 1995), *cert. denied*, 517 U.S. 1136 (1996). In *Correa*, Mrs. Gonzales appeared at the emergency room of Hospital San Francisco presenting chest pains. She was given a number and was asked to wait. After waiting for what was apparently several hours without treatment, Mrs. Gonzales left the hospital without being evaluated and apparently upon referral from the hospital went to a doctor's office. Shortly after arriving at the doctor's office, she collapsed and died. Her family sued the hospital claiming that it had not provided an appropriate medical screening to Mrs. Gonzales when she appeared at the hospital emergency room. Mrs. Gonzales's family did not, and in fact they could not, offer proof that the hospital acted with an improper motive. The First Circuit concurred with the reasoning of both *Power* and *Gatewood* and held that EMTALA does not impose a motive requirement. It noted that *Cleland*,

> required there to be *some* motive-whether or not economic-for the disparate treatment. . . . Other courts have declined to follow the Sixth Circuit's lead in this respect, . . . and we agree that the range of improper motives available under the *Cleland* standard "is so broad as to be no limit at all, and as a practical matter amounts to not having a motive requirement."

Id. at 1194 n.9.

While not specifically citing the Sixth Circuit's position or addressing a claim that improper motive must be shown, four other circuits have considered "appropriate medical screening" cases and have reached conclusions that are inconsistent with the Sixth Circuit's construction of EMTALA. The conclusion of those courts has been that the appropriateness of a medical screening examination under EMTALA depends not on the motivation with which the hospital acted in failing to follow standard screening procedures but, instead, on whether the screening examination is even-handedly administered to all patients whose medical conditions are in relevant respects similar. *See Eberhardt v. City of Los Angeles*, 62 F.3d 1253, 1258 (9th Cir. 1995) ("[T]he test is whether the challenged procedure was identical to that provided similarly situated patients."); *Repp v. Anadarko Mun. Hosp.*, 43 F.3d 519 (10th Cir. 1994) ("[A] hospital satisfies the requirements of § 1395dd(a) if its standard screening procedure is applied uniformly to all patients in similar medical circumstances."); *Holcomb v. Monahan*, 30 F.3d 116, 117 (11th Cir. 1994) ("As long as a hospital applies the same screening procedures to indigent patients which it applies to paying patients, the hospital does not violate [Section 1395dd(a)]."); *See also Marshall v. East Carroll Parish Hosp. Serv. Dist.*, 134 F.3d 319, 322 (5th Cir. 1998) ("[A]n EMTALA 'appropriate medical screening examination' is not judged by its proficiency in accurately diagnosing the patient's illness, but rather by whether it was

performed equitably in comparison to other patients with similar symptoms.").[12]

The universal rejection of the Sixth Circuit's creation of an improper motive requirement under EMTALA makes it abundantly clear that other courts find no such limiting language in EMTALA and recognize that they cannot unilaterally change the plain reading of the statute.

CONCLUSION

The judgment of the Court of Appeals for the Sixth Circuit should be reversed. This case should be remanded to the United States District Court for the Western District of Kentucky for a trial on the merits of Johnson's claim that Humana violated the provisions of EMTALA.

Respectfully submitted on July 23, 1998,

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[12] The question of whether these circuits are correct in using an "even-handedness" standard is not before the Court and not necessary to the decision in this case. Although it is not apparent from the plain reading of EMTALA that a claimant must prove "even-handedness," it *is* apparent that Humana departed substantially from its ordinary practice in discharging and transferring Johnson.

42 U.S.C. § 1395dd. Examination and treatment for emergency medical conditions and women in labor

(a) Medical screening requirement. In the case of a hospital that has a hospital emergency department, if any individual (whether or nor eligible for benefits under this title [42 USCS §§ 1395 et seq.]) comes to the emergency department and a request is made on the individual's behalf for examination or treatment for a medical condition, the hospital must provide for an appropriate medical screening examination within the capability of the hospital's emergency department, including ancillary services routinely available to the emergency department, to determine whether or not an emergency medical condition (within the meaning of subsection (e)(1)) exists.

(b) Necessary stabilizing treatment for emergency medical conditions and labor. (1) In general. If any individual (whether or not eligible for benefits under this title [42 USCS §§ 1395 et seq.]) comes to a hospital and the hospital determines that the individual has an emergency medical condition, the hospital must provide either –

(A) within the staff and facilities available at the hospital, for such further medical examination and such treatment as may be required to stabilize the medical condition, or

(B) for transfer of the individual to another medical facility in accordance with subsection (c).

(2) Refusal to consent to treatment. A hospital is deemed to meet the requirement of paragraph (1)(A) with respect to an individual if the hospital offers the individual the further medical examination and treatment described in that paragraph and informs the individual

(or a person acting on the individual's behalf) of the risks and benefits to the individual of such examination and treatment, but the individual (or a person acting on the individual's behalf) refuses to consent to the examination and treatment. The hospital shall take all reasonable steps to secure the individual's (or person's) written informed consent to refuse such examination and treatment.

(3) Refusal to consent to transfer. A hospital is deemed to meet the requirement of paragraph (1) with respect to an individual if the hospital offers to transfer the individual to another medical facility in accordance with subsection (c) and informs the individual (or a person acting on the individual's behalf) of the risks and benefits to the individual of such transfer, but the individual (or a person acting on the individual's behalf) refuses to consent to the transfer. The hospital shall take all reasonable steps to secure the individual's (or person's) written informed consent to refuse such transfer.

(c) Restricting transfers until individual stabilized. (1) Rule. If an individual at a hospital has an emergency medical condition which has not been stabilized (within the meaning of subsection (e)(3)(B)), the hospital may not transfer the individual unless –

(A)(i) the individual (or a legally responsible person acting on the individual's behalf) after being informed of the hospital's obligations under this section and of the risk of transfer, in writing requests transfer to another medical facility,

(ii) a physician (within the meaning of section 1861(r)(1) [42 USCS § 1395x(r)(1)]) has signed a certification that [,] based upon the

information available at the time of transfer, the medical benefits reasonably expected from the provision of appropriate medical treatment at another medical facility outweigh the increased risks to the individual and, in the case of labor, to the unborn child from effecting the transfer, or

(iii) if a physician is not physically present in the emergency department at the time an individual is transferred, a qualified medical person (as defined by the Secretary in regulations) has signed a certification described in clause (ii) after a physician (as defined in section 1861(r)(1) [42 USCS § 1395x(r)(1)]), in consultation with the person, has made the determination described in such clause, and subsequently countersigns the certification; and

(b) the transfer is an appropriate transfer (within the meaning of paragraph (2)) to that facility.

A certification described in clause (ii) or (iii) of subparagraph (A) shall include a summary of the risks and benefits upon which the certification is based.

(2) Appropriate transfer. An appropriate transfer to a medical facility is a transfer –

(A) in which the transferring hospital provides the medical treatment within its capacity which minimizes the risks to the individual's health and, in the case of a woman in labor, the health of the unborn child;

(B) in which the receiving facility –

(i) has available space and qualified personnel for the treatment of the individual, and

(ii) has agreed to accept transfer of the individual and to provide appropriate medical treatment;

(C) in which the transferring hospital sends to the receiving facility all medical records (or copies thereof), related to the emergency condition for which the individual has presented, available at the time of the transfer, including records related to the individual's emergency medical condition, observations of signs or symptoms, preliminary diagnosis, treatment provided, results of any tests and the informed written consent or certification (or copy thereof) provided under paragraph (1)(A), and the name and address of any on-call physician (described in subsection (d)(1)(C)) who has refused or failed to appear within a reasonable time to provide necessary stabilizing treatment;

(D) in which the transfer is effected through qualified personnel and transportation equipment, as required including the use of necessary and medically appropriate life support measures during the transfer; and

(E) which meets such other requirements as the Secretary may find necessary in the interest of the health and safety of individuals transferred.

(d) Enforcement. (1) Civil monetary penalties. (A) A participating hospital that negligently violates a requirement of this section is subject to a civil money penalty of not more than $50,000 (or not more than $25,000 in the case of a hospital with less

than 100 beds) for each such violation. The provisions of section 1128A [42 USCS § 1320a-7a] (other than subsections (a) and (b)) shall apply to a civil money penalty under this subparagraph in the same manner as such provisions apply with respect to a penalty or proceeding under section 1128A(a) [42 USCS § 1320a-7a(a)].

(B) Subject to subparagraph (C), any physician who is responsible for the examination, treatment, or transfer of an individual in a participating hospital, including a physician on-call for the care of such an individual, and who negligently violates a requirement of this section, including a physician who –

(i) signs a certification under subsection (c)(1)(A) that the medical benefits reasonably to be expected from a transfer to another facility outweigh the risks associated with the transfer, if the physician knew or should have known that the benefits did not outweigh the risks, or

(ii) misrepresents an individual's condition or other information, including a hospital's obligations under this section,

is subject to a civil money penalty of not more than $50,000 for each such violation and, if the violation is gross and flagrant or is repeated, to exclusion from participation in this title [42 USCS §§ 1395 et seq.] and State health care programs. The provisions of section 1128A [42 USCS § 1320a-7a] (other than the first and second sentences of subsection (a) and subsection (b)) shall apply to a civil money penalty and exclusion under this subparagraph in the same

manner as such provisions apply with respect to a penalty, exclusion, or proceeding under section 1128A(a) [42 USCS § 1320a-7a(a)].

(C) If, after an initial examination, a physician determines that the individual requires the services of a physician listed by the hospital on its list of on-call physicians (required to be maintained under section 1866(a)(1)(I) [42 USCS § 1395cc(a)(1)(I)]) and notifies the on-call physician and the on-call physician fails or refuses to appear within a reasonable period of time, and the physician orders the transfer of the individual because the physician determines that without the services of the on-call physician the benefits of transfer outweigh the risks of transfer, the physician authorizing the transfer shall not be subject to a penalty under subparagraph (B). However, the previous sentence shall not apply to the hospital or to the on-call physician who failed or refused to appear.

(2) **Civil enforcement. (A) Personal harm.** Any individual who suffers personal harm as a direct result of a participating hospital's violation of a requirement of this section may, in a civil action against the participating hospital, obtain those damages available for personal injury under the law of the State in which the hospital is located, and such equitable relief as is appropriate.

(B) Financial loss to other medical facility. Any medical facility that suffers a financial loss as a direct result of a participating hospital's violation of a requirement of this section may, in a civil action against the participating hospital, obtain those damages available for financial loss, under the law of the State in which the

hospital is located, and such equitable relief as is appropriate.

(C) Limitations on actions. No action may be brought under this paragraph more than two years after the date of the violation with respect to which the action is brought.

(3) Consultation with peer review organizations. In considering allegations of violations of the requirements of this section in imposing sanctions under paragraph (1), the Secretary shall request the appropriate utilization and quality control peer review organization (with a contract under part B of title XI [42 USCS §§ 1320c et seq.]) to assess whether the individual involved had an emergency medical condition which had not been stabilized, and provide a report on its findings. Except in the case in which a delay would jeopardize the health or safety of individuals, the Secretary shall request such a review before effecting a sanction under paragraph (1) and shall provide a period of at least 60 days for such review.

(e) **Definitions.** In this section:

(1) The term "emergency medical condition" means –

(A) a medical condition manifesting itself by acute symptoms of sufficient severity (including severe pain) such that the absence of immediate medical attention could reasonably be expected to result in –

(i) placing the health of the individual (or, with respect to a pregnant woman, the health of the woman or her unborn child) in serious jeopardy,

(ii) serious impairment to bodily functions, or

(iii) serious dysfunction of any bcdily organ or part; or

(B) with respect to a pregnant women [woman] who is having contractions –

(i) that there is inadequate time to effect a safe transfer to another hospital before delivery, or

(ii) that transfer may pose a threat to the health or safety of the woman or the unborn child.

(2) The term "particpating hospital" means hospital that has entered into a provider agreement under section 1866 [42 USCS § 1395cc].

(3)(A) The term "to stabilize" means, with respect to an emergency medical condition described in paragraph (1)(A), to provide such medical treatment of the condition as may be necessary to assure, within reasonable medical probability, that no material deterioration of the condition is likely to result or occur during the transfer of the individual from a facility, or, with respect to an emergency medical condition described in paragraph (1)(B), to deliver (including the placenta).

(B) The term "stabilized" means, with respect to an emergency medical condition described in paragraph (1)(A), that no material deterioration of the condition is likely, within reasonable medical

probability, to result from or occur during the transfer of the individual from a facility, or, with respect to an emergency medical condition described in paragraph (1)(B), that the woman has delivered (including the placenta).

(4) The term "transfer" means the movement (including the discharge) of an individual outside a hospital's facilities at the direction of any person employed by (or affiliated or associated, directly or indirectly, with) the hospital, but does not include such a movement of an individual who (A) has been declared dead, or (B) leaves the facility without the permission of any such person.

(5) The term "hospital" includes a rural primary care hospital (as defined in section 1861(mm)(1) [42 USCS § 1395x(mm)(1)]).

(f) Preemption. The provisions of this section do not preempt any State or local law requirement, except to the extent that the requirement directly conflicts with a requirement of this section.

(g) Nondiscrimination. A participating hospital that has specialized capabilities or facilities (such as burn units, shock-trauma units, neonatal intensive care units, or (with respect to rural areas) regional referral centers as identified by the Secretary in regulation) shall not refuse to accept an appropriate transfer of an individual who requires such specialized capabilities or facilities if the hospital has the capacity to treat the individual.

(h) No delay in examination or treatment. A participating hospital may not delay provision of an appropriate

medical screening examination required under subsection (a) or further medical examination and treatment required under subsection (b) in order to inquire about the individual's method of payment or insurance status.

(i) **Whistleblower protections.** A participating hospital may not penalize or take adverse action against a qualified medical person described in subsection (c)(1)(A)(iii) or a physician because the person or physician refuses to authorize the transfer of an individual with an emergency medical condition that has not been stabilized or against any hospital employee because the employee reports a violation of a requirement of this section.

(Aug. 14, 1935, ch. 531, Title XVIII, Part C, § 1867, as added April 7, 1986, P. L. 99-272, Title IX, Subtitle A, Part 1, Subpart B, § 9121(b), 100 Stat. 164; Oct. 21, 1986, P. L. 99-509, Title IX, Subtitle D, Part 1, § 9307(c)(4), 100 Stat. 1996; Oct. 22, 1986, P. L. 99-514, Title XVIII, Subtitle C, Ch 1, § 1895(b)(4), 100 Stat. 2933; Dec. 22, 1987, P. L. 100-203, Title IV, Subtitle A, Part 1, § 4009(a)(1), (2), 101 Stat. 1330-56; July 1, 1988, P. L. 100-360, Title IV, Subtitle B, § 411(b)(8)(A)(i), 102 Stat. 772; Oct. 13, 1988, P. L. 100-485, Title VI, § 608(d)(18)(E), 102 Stat. 2419; Dec. 19, 1989, P. L. 101-239, Title VI, Subtitle A, Part 1, Subpart A, § 6003(g)(3)(D)(xiv), Part 3, Subpart B, § 6211(a)-(h), 103 Stat. 2154, 2245; Nov. 5, 1990, P. L. 101-508, Title IV, Subtitle A, Part 1, § 4008(b)(1)-(3)(A), Part 3, § [4207]4027(a)(1)(A), (2), (3), (k)(3), 104 Stat. 1388-44, 1388-117, 1388-124.)